See: Designed for Life: Welsh Guidelines for the transfer of the critically ill adult

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Pre-transfer Checklist. (For transfers to another hospital)

1. VENTILATED PATIENTS *(non ventilated → proceed to section 2)*
☐ Check ETT position and secured.
☐ $ETCO_2$ sampling after HME filter in circuit.
☐ Check PEEP and Tidal volume *(target 6-8ml/kg).*
☐ Check ABG on transfer ventilator *(adjust rate to keep $PaCO_2$ in target).*
☐ Thoracostomies or Chest Drains required? *(All suspected or confirmed pneumothoraces)*
☐ Oesophageal temperature probe in situ. Target temperature =
☐ Check pupils. Protect eyes *(as appropriate).*
☐ Neuroprotection (if applicable) as per guidelines (see reverse)
2. ALL PATIENTS
☐ Patient ID wristband x 2 and **visually checked**.
☐ Accepted by receiving unit specialty and ward / ITU.
☐ Transfer team appropriately dressed and briefed.
☐ Patient warming as appropriate with Blizzard blanket or alternative.
☐ Notes, investigation results (+/- referral letter) copied & patient belongings packaged.
☐ Transfer documentation started.
☐ Vascular access x 2 – all flushed, secured and accessible with 3 way tap & extension *(Helicopter → Left side, Road transfer → Right side).*
☐ Snag risk minimised (lines and fluids).
☐ NIBP in situ on regular cycles.
☐ Arterial line and transducer accessible *(as appropriate).*
☐ Monitor/syringe drivers/ventilator secured appropriately.
☐ Check position of all chest drains, lines, ETtube, ng tube *(visual or on CXR / CT as appropriate)*
☐ Catheter checked and bag emptied *(as appropriate).*
☐ Anti-emetic given *(as appropriate).*
3. EQUIPMENT TO HAVE IMMEDIATELY ACCESSIBLE
☐ Emergency Anaesthesia Drugs.
☐ Intubation pack including BVM + LMA or iGel.
☐ Primary response bag & monitoring: ECG, SaO_2, NIBP, $ETCO_2$, pen-torch for Pupils.
☐ Muscle relaxant *(double predicted amount drawn up).*
☐ Sedation *(double predicted amount drawn up).*
☐ Vasopressors *(Infusion plus bolus if required).*
4. EQUIPMENT & DRUGS
☐ Visually check battery on all equipment being used.
☐ Sufficient oxygen *(double predicted amount).*
☐ Blood box (if applicable) plus paperwork & blood bank informed.
☐ Mannitol or hypertonic Saline *(as appropriate).*
5. COMMUNICATION
☐ Air or road crew – discuss transport plans including returning to base.
☐ Team have phone with numbers of consultant, receiving unit & Money.
☐ Relatives briefed; information and directions given.
☐ Receiving hospital informed of ETA. ***Remember to update ETA approx 20 minutes pre-arrival.***

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